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"Le geste de Pinel": The History of a Psychiatric Myth

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The French do not know the founder of psychiatry in their own country. All that most textbooks tell us about Philippe Pinel (1745–1826) is that he was the first to liberate the insane from their chains, at Bicêtre Hospice in Paris at the height of the French Revolution. Yet recent scholarship and documentary evidence definitively discredit that legend, known in France as "Pinel's gesture." The facts do not support it, yet the legend persists. It has a life of its own because, like all fairytales, it hides a deeper meaning. This chapter explores the reasons why a crudely manufactured myth has overshadowed Pinel's innovative and creative achievement, leading one of the foremost French historians of psychiatry of our own age to announce a new attitude when he writes: "Our generation has stopped adoring [Pinel] and has begun to read him."[1]

More than any other public medical figure 200 years ago, Pinel offered society assurance that mental illness is treatable and sometimes curable. To this end he advocated close and extended observation of, and carefully structured conversations with each patient so as to understand the natural history of the illness and to arrive at individual case histories. He used the precipitating event, the duration of each illness, and his analysis of the affected mental faculty to fashion a diagnosis. He then subdivided his patients into categories and used the accommodations and facilities of the asylum to further therapy. Pinel insisted on the prohibition of physical violence against patients and on a drastic reduction in bloodletting for therapeutic purposes; he was skeptical of drugs, preferring natural remedies. He paid serious attention to the complaints of the poor as well as the rich and stayed at the sickbed to help his indigent patients die. Owing to an unfortunate and thoughtless translation into English, his psychologically sensitive approach to the mentally ill has become known as "moral treatment." The fact is that he attempted to raise the rejected madmen and madwomen housed in the kingdom's public institutions to the dignified status of medical patients in the newly formed French republic.[2] In that sense, and in that sense only, did he liberate the insane men at Bicêtre from their chains.

Pinel has remained controversial: He was a public servant, clinician, author, and professor living in Paris during the Revolution and Empire, and therefore his every

word and action were intertwined with the political events of twenty-five turbulent years. Even though he had little interest in politics and never took an active part, he was forced to make choices, express allegiances, and side with or against successive ruling powers. Political regimes have therefore claimed or rejected him, historians have faulted him for loyalties or fickleness, and the informed public has judged his person, his work, and even his contribution to medicine in political terms. Certain of his critics seem determined to disparage the man in every possible way.

In order to understand the long life of the myth, it will be useful to view Pinel in relation to four general themes of great importance to him, to psychiatry, and to French history. These themes are the Catholic religion, the revolution in the medical sciences at the end of the Enlightenment, French educational politics in the Revolutionary and Imperial era, and the impact of anatomo-pathology on psychiatry in the early decades of the nineteenth century. But the reader should first be introduced to the myth.

In *Le sujet de la folie* and some complementary articles, Gladys Swain, the brilliant French psychiatrist, has shown how Dr. Scipion Pinel (1795–1859), the professor's eldest son, and Dr. J. E. D. Esquirol (1772–1840), his favorite pupil and spiritual heir, began the process of constructing a sentimental symbol.[3] In 1823, with Pinel increasingly incapacitated by successive strokes, Scipion published a brief piece on ''The removal of the chains,'' supposedly extracted from his father's papers. ''Pinel's gesture'' is here described for the first time.[4] Here towers the tall, muscular Chevingé, liberated from chains by Pinel, the lifelong bodyguard and servant who presumably saved the doctor from deranged inmates more than once.[5] The main episode in Scipion's story is Pinel's defense of his patients' right to personal freedom at Bicêtre in 1794.

In an embellished version presented to the Royal Academy of Medicine in 1836, Scipion portrayed Pinel confronting the cruel, paraplegic Georges Couthon, a member of the autocratic Committee of Public Safety, who came to inspect the situation of the chained ''wild beasts.''[6] In the story, Pinel heroically faces Couthon, demanding that all the unfairly restrained madmen be instantly freed. He risked his life in defying the powerful Jacobin. But nothing in Couthon's biography, Pinel's writings, or contemporary accounts corroborates this visit. In fact, Couthon seems to have been out of town at that time.[7]

Scipion's motivation in publicizing his father's fearless attitude was undoubtedly complex. He was surely keen on diverting attention from the father's emphasis on psychologic factors in illness. That emphasis was embarrassing to a son who was convinced that mental illness is caused by diseased brains. Scipion, fifty years his father's junior, lived in a different scientific, cultural, and political world. But we also know that Scipion had money troubles and that he borrowed heavily, using his father's property and good name to obtain credit; he even appropriated Pinel's library while the father was still alive. The total lack of letters or other personal documentation forces us to leave the two men's feelings toward each other to conjecture.[8]

The same is true of Pinel's relationship to Esquirol. It suited the ambitious younger man to see the image of his teacher reduced to a symbolic gesture. Pinel had been his mentor, sponsor of his private mental institution since 1802, president of his thesis committee in 1805, his chief at the Salpêtrière from 1811 to 1825. They collaborated closely until Esquirol was a mature man in his fifties. Yet Pinel, sounding both proud and patronizing, continued publicly to call Esquirol his ''student.''

In 1817, under the restored Bourbon monarchy, Esquirol began offering clinical lectures on mental illness at the Salpêtrière, and he took the first step in a lifelong course of action to win political attention for the mentally ill. At the behest of the minister of internal affairs, he undertook a nationwide survey, visiting all the institutions throughout France where mental patients were then confined. Esquirol painted a dramatic picture:

> I have seen them naked, clad in rags, having but straw to shield them from the cold humidity of the pavement where they lie. I have seen them coarsely fed, lacking air to breathe, water to quench their thirst, wanting the basic necessities of life. I have seen them at the mercy of veritable jailers, victims of their brutal supervision. I have seen them in narrow, dirty, infested dungeons without air or light, chained in caverns where one would fear to lock up the wild beasts that luxury-loving governments keep at great expense in their capitals.[9]

Pinel is not mentioned in this text, which implies that his reforms had remained fruitless. The need for a rescuer was evident and urgent.

Esquirol claimed that role for himself. In 1822 he was appointed inspector general of medical faculties, and in 1825 medical director of Charenton Hospice. He became the main architect of the national law of 1838 that instituted departmental asylums for all needy French mental patients and that is still in force today. It is surely significant that the only important formative role by Pinel that Esquirol still acknowledged in the 1830s was that of Pinel's famous book on mental alienation. But in the revised version of the myth that appears in Esquirol's *Treatise on Mental Maladies* of 1838, Pinel, referred to as the "learned doctor," was seen as a tool of anti-governmental rebellion. We read: "The ideas of the time perverted the importance of breaking the chains that degraded and troubled the madmen at Bicêtre. The success of the learned doctor and friend of misfortune turned into a trophy for the agitators."[10] Esquirol the royalist dissociated himself from Pinel the republican.[11]

Had the myth of the chainbreaker been merely the work of two envious "sons," it could not have prevailed for over a century. To understand its deep meaning for other Frenchmen, we need to explore the main themes upon which the myth touches. The first was the Catholic religion: We must analyze its relationship to Pinel and to the nascent psychiatric specialty.

In the region between Toulouse, Albi, and Castres where Pinel spent the first twenty-eight years of his life, the religious past is still a major concern. Even today, conversation turns easily to the cruel suffering of the people during the Albigensian Crusade, during the religious wars of the Reformation, or after the revocation of the Edict of Nantes. Pinel's family was Catholic, but his mother's ancestors came from Protestant Castres: Her parents converted when the Edict was revoked in 1685.[12] Local Protestants did not emigrate in large numbers but continued to inhabit Pinel's native region, shielded by their Catholic neighbors.[13] As a boy, Pinel learned the need for tolerance and for accommodation with powerful political and religious forces. He learned that lesson mainly from the village teacher, Jean Pierre Gorsse (173?–1772), who became a family friend and a role model for the boy. Gorsse had graduated from the Jesuit college at Albi; but by the time Pinel was ready for college, the Jesuits had been expelled from France. Through this teacher's influence Pinel was awarded a scholarship to attend the seminary of the Fathers of the Christian Doctrine at nearby Lavaur. To accept the

scholarship, Pinel needed to receive the tonsure and a recently discovered document indeed tells us that he did.[14] Pinel studied as a tonsured clerk, free of charge, until he completed the Master of Arts degree. He then prepared for the doctorate in theology and it is only around the time of his twenty-fifth birthday that he switched to medicine. Critics from the political Right criticize Pinel bitterly for this ''treason,'' for having ''thrown the cassock into the flames.'' Critics from the Left welcome the change. Both identify psychiatry with left-wing politics.

Did Pinel transfer any aspect of his religious upbringing to his medical teaching and practice? In his writings he never adduced the authority of the Old or New Testament, any saint, dogma, or belief. Nor did he ever utter any moral judgment of a patient that might indicate his own position with regard to Catholic dogma. What Pinel did share with the Catholic faith was a lifelong commitment to help the suffering poor. He served for thirty years as physician-in-chief of Salpêtrière Hospice, caring for indigent, old, and sick women patients. He often stayed at their bedside to help them die. In doing so, he brought the finest humane aspect of Catholic charity to clinical medicine. He also developed the individual doctor-patient relationship into a long-term collaborative undertaking. Swain discovered that the German philosopher Georg Wilhelm Friedrich Hegel appreciated the fact that Pinel's concept of ''*manie périodique*'' offered the therapist the opportunity to gain the patient's confidence and over time to establish a collaborative therapeutic relationship during the patient's lucid intervals.[15] Pinel's use of repeated, probing, personal conversations with his mental patients grew into an important part of ''psychologic treatment.'' His long years of experience with the personal advice and guidance lavished by the Fathers of the Christian Doctrine on their young students leads one to conclude that nascent psychiatry in France owed an important debt to Pinel's clerical past.

Pinel's serious involvement with science, particularly natural history, is another aspect of his work that belies the simpleminded heroism of the myth. Pinel came to Bicêtre after years of work in zoology at the Paris Botanical Gardens, work that resulted in eight important papers published in the *Journal de physique* and led to his election to the Academy of Sciences in the section for anatomy and zoology in 1803. His work is based on comparative anatomy and on the close observation of animal behavior. It also shows a sustained interest in taxonomy and classification.[16] Pinel applied these interests to the mentally ill men at Bicêtre whose health became his main responsibility once he was appointed as ''*médecin des infirmeries*.'' For nineteen months he spent many hours every day observing the behavior of each individual patient and he repeatedly talked to each man so as to understand his personal history. His notes then enabled him to chart the natural history of the disease and arrive at a diagnosis. To depict Pinel as an idealistic and somewhat simpleminded liberator is to ignore historic facts.

The third, and central, part of Pinel's thought and work, trivialized or simply ignored by the advocates of the myth, concerns the teacher and writer. He elaborated his clinical method in three books that complement each other. For his students he wrote a textbook, the *Philosophic Nosography*,[17] presenting a classificatory framework of all diseases. The *Medico-Philosophic Treatise on Mental Alienation* explicated a part of the nosologic picture, namely the nervous illnesses, then called ''neuroses.''[18] And in *Clinical Medicine* Pinel illustrated how he combined attention to physical and psychologic illness at the sickbed, integrating the perceptions, diagnoses, and therapy of the psychi-

atrist and physician into a humane public medicine.[19] This exceptional patient-centered approach impressed Pinel's contemporaries, who often stressed his "kindness." The students prized his skill in honing their clinical acumen.

The *Treatise on Insanity* became world-famous. The *Nosography*, though widely used for at least twenty years in six successive editions, did not stand the test of time. We shall see that critics attacked Pinel's classification, particularly his first class of diseases, the "essential fevers." In fact the initial attack, by an adherent of John Brown (1735–1788), the advocate of "excitement" as the clue to illness and health, appeared soon after the *Nosography*'s publication.[20]

But it is the last phase of Pinel's life, when the Royalist government dismissed him from the medical faculty, when the idol of the young, François Broussais (1772–1838), ridiculed the *Nosography*, when the medical thesis of A. L. J. Bayle (1799–1858) seemed to demonstrate the reliability of the anatomo-pathologic method even in psychiatry, that Pinel's credit fell, and the myth of the liberator established itself. We now turn to the political fortunes of the myth.

Politics was an essential element of the myth. It transformed Pinel's medical message into a controversial claim because he raised questions in the public mind about the rights of the mentally ill, their involuntary confinement, their claim of innocence for crimes by reason of insanity—questions involving the Rights of Man. "Man is born free, yet everywhere he is in chains," Jean Jacques Rousseau had written at the beginning of the *Social Contract*. These chains became identified with those shackling the insane—an identification that transformed Pinel into a political revolutionary.

In nineteenth-century France, the politics of the Rights of Man reached far beyond the struggle for freedom of assembly, speech, and religion. Rather, the fight for individual freedom became part of the confrontation between the Catholic religion and Science, including medical science, and extended from Napoleon's Concordat with the Holy See in 1802 to the separation of state and church in 1905. No nineteenth-century French scientist or physician could escape involvement in that duel: whether it was François Vincent Raspail, the microscopist, who languished in jail in the 1830s as president of the Society for the Rights of Man, or Ernest Renan who lost his chair at the Collège de France in 1862 because he published a factual biography of Jesus, or Claude Bernard who was given a state funeral in 1878 so as to preclude a solemn commemoration of Pope Pius IX—great scientific figures were pushed or used for political ends throughout the nineteenth century. Pinel was no exception.

The battle between Left and Right, claiming the brilliant clinician and learned doctor as one of theirs, began while Pinel was still alive. The regime of Charles X was evidently frightened by student agitation, in the early 1820s, in favor of broader suffrage and a constitution guaranteeing freedom of assembly, expression, religion, and the press. The University of Paris, particularly its medical faculty, was a hotbed of political protest in which Pinel's son Charles (1802–1871), a law student, was involved. Under a pretext, the reactionary minister of education, Jacques Joseph Guillaume Pierre Corbière, closed and then purged the medical faculty on 21 November 1822. On 2 February 1823, Pinel was placed on emeritus status, on half-pay, together with ten eminent colleagues also

perceived as liberal heralds of revolution.[21] The Bourbons—the Right—thereby rejected Pinel.

Three years later, politics intruded into Pinel's very funeral at the Père Lachaise cemetery. Etienne Geoffroy St. Hilaire spoke, representing the Academy of Sciences,[22] then Etienne Pariset, for the Academy of Medicine,[23] followed by Léon Louis Rostan, for the Salpêtrière staff. ''When no one came forward for the medical faculty,'' wrote the anonymous author of a ''Notice'' in the *Archives générales de médecine*, ''M. Cruveilhier [Jean Cruveilhier, the distinguished anatomist] made his way through the crowd and improvised a speech full of warmth and feeling. It is said that an eyewitness, seeing the Faculty thus remain silent among this justified and universal sadness, remarked that [the Faculty] was attempting to dismiss Pinel a second time.''[24] Cruveilhier, a member of the *Congrégation*, the arm of Catholic reaction, had let his personal decency and admiration for Pinel silence his political allegiance. Not so Cruveilhier's teacher, the king's powerful first surgeon, Guillaume Dupuytren, who concluded an obituary ''Notice'' in the *Journal des Débats*, claiming Pinel for the Right: ''He was a Christian,'' wrote Dupuytren, ''and of his free will he died a Christian.''[25] Pinel was too important a figure to abandon to the Left. (Whether Pinel in fact received extreme unction has not yet been established.)

As for the abolition of chains and the claim of priority so often made for (not by) Pinel, the documents give a clear picture—more precise than historians have hitherto realized. Since so much ink has flowed on this subject of chains, and they are the theme of the self-perpetuating myth discussed in this chapter, it is worth establishing a definitive record of the facts and their sources. To begin with, we have testimony about the orderly St. Prix ward for mentally ill men at Bicêtre, run by Jean Baptiste Pussin (1745–1811), soon to become Pinel's assistant. This evidence comes from a wholly trustworthy source, the site visitors of the Duke de La Rochefoucauld-Liancourt's Poverty Committee of the National Assembly who inspected Bicêtre in May 1790. They reported that ''the number of [violent patients] is small and varies with the seasons: only ten out of 270 were chained the day of our visit.''[26]

What is most surprising, given the ubiquitous propaganda for Pinel as the ''chain-breaker,'' is that, when he initially arrived at Bicêtre on 6 August 1793 as ''physician of the infirmaries,'' Pinel accepted the traditional use of chains to restrain the violent insane as a matter of course. In his ''Memoir on Madness'' of 11 December 1794, he mentioned without further comment that three of his patients had been shackled for fifteen, twenty-five, and forty-five years, respectively.[27] It was his talented assistant Pussin who, evidently on his own initiative, first freed the insane men at Bicêtre from their fetters in Prairial, Year V [May–June 1797].[28] At that time Pinel had been gone from Bicêtre for two years. The doctor praised Pussin's exemplary accomplishment in the second edition of the *Treatise* in 1809, mistakenly giving the date ''4 Prairial, Year VI'' for Pussin's historic initiative, adding that he followed his assistant's example ''three years later.''[29]

Both men knew that some mental patients need to be restrained if they become violent and destructive or suicidal, or threaten the lives of others. And all humane persons object to the weight of iron shackles and the unnecessary pain they inflict as well as the degradation they imply. Why Pinel did not take immediate action remains unexplained and reprehensible. When he and Pussin eventually dispensed with chains,

they substituted straitjackets when needed, a fact that is attested to repeatedly in the contemporary official literature. Thus the Paris Hospital Council report of 1803 simply stated that the condition of the mentally ill at Bicêtre "has been improved, quite apart from the abolition of chains."[30] The second report, in 1816, reveals a new interest in the mentally ill. A thirteen-page chapter deals with mental hospices, and Pinel's innovative methods of management and therapy are discussed at length. A separate paragraph concerns Pussin and chains. Since this official statement has hitherto passed unnoticed, it is here translated *in extenso.*

> The idea of chaining the mad so as to dominate and confine them more easily had been adopted in all insane asylums in Paris and all other European establishments. A dedicated, intelligent and courageous supervisor who recently died, M. Pussin, brought about a useful and humane change. His efforts were entirely successful. The excitement of maniacs subsides instead of growing when they are not chained. They are now restrained, even during their most violent attacks, in a straitjacket made of strong cloth, with sleeves longer than the arms that can be tied at their ends. The sleeves can be used to fasten the inmates to their beds, without shackling their limbs.[31]

Pinel's and Pussin's contemporaries attributed a specific role to each man and distinguished clearly between Pinel, who reflected on theory and improved management as well as therapy, and Pussin, who initiated the removal of chains or their replacement with straitjackets. Scipion Pinel must not have known this report or he would have hesitated to publish his tall tale.

While the myth was gradually taking hold, Pinel's scientific teachings were being seriously challenged by young medical investigators. Scientists were now searching for the seat of diseases, in the tradition of Giambattista Morgagni. They analyzed tissues, following Xavier Bichat; explored physiologic functions with François Magendie; sought to establish neurologic localization with the phrenologists Gall and Spurzheim; scanned physiognomy with Camper and Lavater. They relied on physical diagnosis with the help of Auenbrugger's percussion and Laënnec's stethoscope. Eager researchers dissected brains to locate the causes of delirium, depression, retardation, and other mental disturbances. Pinel was looking increasingly old-fashioned.

The first skirmish in the battle to dethrone the old master had occurred as early as 1802. That year Gaspard Laurent Bayle (1774–1816) defended his thesis entitled "Considerations on Nosology, Medical Observation and Practice"[32] before a jury that included Pinel. Bayle's argument directly attacked nosology, and thus the author of the popular *Nosography*, and questioned the concept of species if applied to diseases. Bayle argued that species in medicine are based on subjective criteria and not on objective characteristics like those presented by animals, plants, or minerals. A physician observing a sick person selects signs and symptoms that he believes indicate the presence of a specific illness, then he combines a number of similar cases to conceptualize a species of disease. Thus he uses a double abstraction. Bayle then dismissed Pinel's classification of "discrete" and "confluent" smallpox as two different species of the disease, arguing that they belong to the same species because the inoculation of one can produce the other. And the transmission of such characteristics is a basic scientific criterion for identifying a species.[33]

Fortunately for the historian, Bayle's friend R. T. H. Laënnec was present and he took stenographic notes.[34] Bayle's argumentativeness had annoyed Professor Petit-Radel, so Pinel intervened in a conciliatory tone: ''I feel great esteem for your personal qualities and your learning,'' Pinel told the candidate, ''let us amicably discuss a few points on which we differ,'' and he suggested the two species of smallpox for discussion. Bayle explained his position, driving home the argument that Pinel failed to adhere to the rules of natural history, rules by which he himself taught medicine should be guided. Pinel was multiplying species of diseases unnecessarily, argued Bayle. Pinel defended himself, saying that such specificity was helpful in guiding therapy (which was, of course, beside the point), he interrupted Bayle several times and almost lost his temper. Bayle insisted, with some insolence, that he be heard. ''You are right,'' conceded Pinel, ''everyone must be allowed to state his argument freely.'' The battle was won. In the course of the discussion Pinel made one crucial concession: ''The aim of classification is to aid memory,'' he stated. ''That is the aim of methods in medicine.''

''This discussion remains memorable to many physicians who were Bayle's contemporaries,'' commented Bayle's nephew, A. L. J. Bayle in 1855. The *Nosography* had found its first critic.

Yet it is significant that G. L. Bayle, like so many of Pinel's students, was more indebted to his teacher than he realized. In Bayle's most important work, the *Researches on Pulmonary Phthisis* of 1809–1810, he defined the lesion in the lung as a constantly developing entity, thus incorporating Pinel's vision of the changing clinical reality in the natural history of diseases. Even the critics among Pinel's students were touched by his teaching, and Pinel's students numbered in the thousands.[35]

This was also true for François Joseph Victor Broussais, the combative Breton military doctor who dedicated his thesis on ''hectic fevers'' to the professor in 1803. His book on *Phlegmasias* of 1808 is also faithful to Pinel's and Xavier Bichat's teachings.[36] But Broussais belonged to Esquirol's generation, and by the time he reached prominence as professor at the Val-de-Grâce in 1815, the Paris teaching hospital for military doctors, he had elaborated a new doctrine. He now explained most illnesses as varieties of inflammation, mainly of the gastrointestinal tract. The inflammation was caused by irritation and indicated a disturbance of function and usually a disease he called ''gastro-enteritis.'' Broussais called his doctrine ''physiologic medicine,'' and, in order to make it prevail, he launched a frontal attack on Pinel's classificatory category of ''essential fevers'': Fevers were only symptoms, Broussais rightly maintained. His gift of eloquence and his youthful, aggressive energy (in contrast to the professors at the medical faculty most of whom were aging) attracted the students in droves. Broussais's *Examination of the Widely Adopted Medical Doctrine* of 1816 was a brutal pamphlet. ''Within ten years, [he] made Pinel's *Nosography* age by a century,'' comments Ackerknecht.[37]

The third and most serious scientific challenge to Pinel's preeminence, after those by G. L. Bayle and Broussais, occurred in 1822 when Antoine Laurent Jessé Bayle (1799–1858), Gaspard Laurent's nephew, defended a thesis entitled ''Research on Mental Illness.''[38] Bayle had observed and autopsied almost 200 mental patients under the eye of his mentor A. A. Royer-Collard (1768–1825), the medical director of Charenton Hospice. In his thesis Bayle presented six patient histories. All of these persons had suffered from increasingly severe paralysis and dementia—these illnesses had fre-

Figure 1. Philippe Pinel (1745–1826). "Pinel Orders the Removal of Iron Shackles from the Insane Men at Bicêtre Hospice." Painting by Charles Müller, 1849. Lobby, National Academy of Medicine, Paris.

quently been observed to be concurrent. Bayle now revealed that, on autopsy, the arachnoid membrane in the patients' brains showed increasingly severe inflammation, and he concluded that "chronic arachnoiditis exists and is the cause of a symptomatic mental alienation."[39] (Actually he could not prove a cause, but only a correlation.) He enlarged upon his observations in *Nouvelle doctrine des maladies mentales* in 1825 and *Traité des maladies du cerveau et de ses membranes* in 1826. Bayle's discovery "strengthened somaticist tendencies," comments Ackerknecht, it "was the end of ancient psychiatry."[40]

While the young generation was thus challenging Pinel's leadership, Scipion's story of the myth had been quietly working its way into powerful circles. Immediately after his father's death, in 1826, Scipion provided Georges Cuvier, the permanent secretary of the Academy of Sciences, with the notes for the official eulogy.[41] From there the myth and "Pinel's gesture" found their way into Etienne Pariset's tribute in the Academy of Medicine.[42] In 1836 Scipion read his expanded version of the myth to this academy, as we have already mentioned. As luck would have it, the Academy of Medicine soon thereafter planned to move into permanent quarters, rue des Saints Pères, where it wished to decorate its meeting hall with paintings of heroic medical deeds. Larrey performing surgery on the battlefield was an obvious theme, but how did "Pinel Orders the Removal of Iron Shackles from the Insane Men of Bicêtre" become the second choice?

The documents are somewhat inconclusive. But it is a fact that the secretary of the Academy of Medicine, Dubois d'Amiens, had heard Scipion's paper, and correspondence between him and the ministry of internal affairs indicates that the artist to execute the paintings for the academy, Charles Müller (1815–1892), was easily chosen.[43] How Müller was commissioned to immortalize the Pinel myth we do not know. The Academy—and the paintings—moved to 16, rue Bonaparte in 1902, and now a huge canvas of eighteen feet by eight feet, depicting an event that never happened, greets the visitor in the lobby of the Paris Academy of Medicine. On this canvas a youthful Pinel strides

through some classical buildings, followed by an awed crowd and accompanied by a young associate who can only be Esquirol. Pinel, extending his arm in a commanding gesture, directs an employee (Pussin?) to unlock the irons on the wrists of an emaciated old man.

But history played a strange trick, and the image of Pinel as the herald of liberty, commissioned under the July Monarchy, was not unveiled in the meeting hall of the rue des Saints Pères until after the ''February Revolution'' of 1848, under the Second Republic. An anonymous newspaper article proclaimed ''Pinel's triumph,'' editorializing: ''One day Providence sent Pinel: his great genius brought about this transformation.'' The republic had gained an official medical hero.[44]

Müller's painting was followed in 1856 by a portrait showing Pinel in his professorial robes, ''offered to the Imperial Academy of Medicine by his family, his friends and his students.'' Responsible for this initiative was Dr. Casimir Pinel, the son of Philippe's brother Louis, who signed himself ''Pinel nephew.''[45] In 1856, the academy also acquired a bust of Pinel by P. Robinet, now standing in the lobby under Müller's canvas. These two works of art resulted from acts of familial piety and do not carry a political connotation.

Not so the canvas by Tony Robert-Fleury (1837–1912) that adorns the entrance to the Charcot Library at the Salpêtrière, where it was inaugurated in 1878. Here the Third Republic paid tribute to its hero with a painting entitled ''Pinel Delivering the Insane.''

Figure 2. Philippe Pinel (1745–1826). ''Pinel Delivering the Insane.'' Detail from a painting by Tony Robert-Fleury, 1878. Charcot Library, Salpêtrière Hospital, Paris.

Even though the word "*aliénés*" is in the masculine, the canvas shows a distraught young woman, behind her a female attendant, with a man unlocking a broad girdle that restrains the patient at the waist. In contrast to its counterpart at Bicêtre, this painting has historic veracity, for Pinel did order the chains removed at this hospital. Furthermore, the woman standing behind the girl unquestionably portrays Marguerite Pussin, since 1802 the supervisor of the mentally ill woman;[46] her husband, Jean Baptiste, had the keys to the shackles. Madame Pussin's head is centrally placed in this scene, close to the doctor's and higher than her husband's, which is virtually invisible behind the girl's arm. Madame Pussin's face expresses concern for the young woman and she appears personally involved in the liberation that Pinel has ordered. The artist judged—rightly, in my opinion—that this woman was an important actress in these proceedings.

It is remarkable how often and warmly Pinel praised Madame Pussin. In his "Memoir on Madness" of 1794 he wrote a whole paragraph on the improvement of the food at Bicêtre under her stewardship, praising "the succulent and tasty soup" . . . it had saved his life that year when he was recovering from typhus. Ten years later, in the twenty-four-page foreword to the second edition of *Clinical Medicine*, he stressed four areas of recent improvements at the Salpêtrière, three of them concerning food, laundry, and cleanliness.[47] Pinel repeatedly recorded his appreciation for her skill in managing recalcitrant patients. (There is an obvious parallel between Mme. Pussin and her contemporaries, Mrs. Dunston, the matron at St. Luke's Hospital in London, and Katharine Jepson at the York Retreat as portrayed by Samuel Tuke.)

As for Monsieur Pussin, Pinel's repeated praise made him overbearing—witness the following, hitherto unknown story. We find Pussin, together with Pinel and at least eighty French physicians, surgeons, and pharmacists among the corresponding members of the Société de médecine de Bruxelles in 1808. The exact date of their election cannot be ascertained from the surviving documents.[48] The picturesque popular diarist known as "Père Richard," a longtime employee at Bicêtre, mistook Pussin's membership certificate for a medical degree. He recounts that he himself "handed Pussin his diploma of doctor of the faculty of medicine of Brussels."[49] And, indeed, Pussin appears in the account books of the Salpêtrière as "physician of the madwomen" with a salary of 1,200 francs, half of that of Pinel and 200 francs more than Pinel's medical assistant A. J. Landré-Beauvais.[50] It is surprising that the administration—and Pinel—condoned this arrangement.

In addition to paintings, Republican Paris had other ways of honoring the famous liberator: A *rue Pinel* was inaugurated in the 13th arrondissement on 9 April 1851 (in connection with his birthday on the 20th), a *place Pinel*, also near the Salpêtrière, on 26 February 1867, and finally a postage stamp for 8 francs was issued on 25 January 1958.[51] And the Société médico-psychologique periodically honored the founder of the psychiatric profession in France, on the 100th and 150th anniversaries of his death.[52] Great specialists and others gathered on these occasions to offer paeans of praise in which the myth of the liberator provided a predominant theme. One exception was an excellent paper on the Wild Boy of Aveyron presented in 1976 by T. Gineste and R. Mises where the empathy and creative involvement of Dr. J. M. G. Itard, "Victor's" teacher (and Pinel's student), overshadows the professor's rejection of the boy as an incurable idiot.[53] A bicentennial celebration of the meeting between Pinel and Pussin is being planned at Bicêtre Hospital in June 1993.

The crowning effort of the Société médico-psychologique to honor the founder of

psychiatry in France resulted in a statue of Pinel by Ludovic Durand erected near the entrance portal to the Salpêtrière. Paid for by public subscription, it portrays a youthful, elegant man holding links of a broken chain. At his feet cower two young girls, overcome with emotion, who look up at him in surprised gratitude. Statues of *Science* and *Bienfaisance* flank the liberator. And to underscore the political meaning of the event, the Society unveiled the statue on *13* July 1885, thus linking the breaking of chains with the storming of the Bastille.[54]

Thus the memory of Philippe Pinel has been kept alive in a historiographic controversy that would have perplexed him. Pinel himself is partly responsible, because his writings do not provide precise answers regarding either his politics or his religious convictions. He accepted hospital appointments, professorships, a seat in the Academy of Sciences, the Legion of Honor, and the Cross of St. Michael from the successive Jacobin, Thermidorian, Napoleonic, and Bourbon regimes. He served as consulting physician to Napoleon from 1808 on. He personally brought his twelve-year-old son to be baptized in 1807, yet four years later he buried his wife under a tombstone without a cross. He became indifferent to religion after leaving the study of theology in April 1770, and disgusted with politics after witnessing the death of Louis XVI under the guillotine on 21 January 1793.

It should by now be clear that the controversy over the Pinel myth conceals a much more important debate over this innovator's historic importance and influence. Indeed, in the medical field, two different approaches to mental illness have confronted each other ever since the defense of A. L. J. Bayle's thesis in 1822. Edward Brown called them ''psychiatry's research programs.''[55] Neuroanatomists who seek to understand the pathology of the human mind by studying the dead brain have been vying with the adherents to Pinel's approach, the advocates of psychologic investigation and treatment. In the 1820s, the young generation was swayed by the rise of a positivist and solidist fashion in medical theory and practice. Meanwhile, the myth of the chainbreaker flourished. Pinel, the innovative clinician, was forgotten and Esquirol became known as the real founder of psychiatry in France. The psychologic approach to mental illness did not again become popular in France until the era of Charcot.

Paradoxically, the strongest recent criticism of Pinel and of the mental hospital comes from sociologists, many of them Marxists, spearheading ''anti-psychiatry.'' These thinkers, brilliantly led by Michel Foucault and Klaus Doerner, see physicians (and psychiatrists in particular) as members of the bourgeois establishment who have, since the early nineteenth century, used huge numbers of indigent patients for research and experiment. According to this argument, these doctors have exploited these patients for their own careers or for profit while claiming the advancement of knowledge as their avowed goal. The doctors turned the hospitals into ''curing machines,'' a depersonalization of the patient set in, the profession becoming increasingly absorbed with diseases, while ''the sick-man gradually disappeared from the doctor's consciousness.''[56] In the asylum, according to Foucault, the director turned into a ''medical personage'' who sits in perpetual judgment over the inmates.[57] The ''liberation'' of the insane from their chains may not have given them liberty after all. Jacques Postel and David Allen explore anti-psychiatry later in this book.

An American admirer of Foucault, Jan Goldstein, has recently written an excellent book on *The French Psychiatric Profession in the Nineteenth Century*. Esquirol is the

hero of her story. Goldstein designates the so-called moral treatment as the "originative psychiatric paradigm" and argues that Pinel derived this from Pussin and other "concierges" of mental institutions. These men were "charlatans" as defined by the *Encyclopédie*. Pinel's therapeutic method, his essential contribution to psychiatry, according to Goldstein, thus "derived from charlatanistic practice." This derogatory interpretation further strengthens the case for reducing Pinel to a mere chainbreaker.[58]

The myth has thus served to obscure Pinel's real achievements in the field he called mental alienation. More than anyone else, he is responsible for transforming French society's perception of the mad into that of sick and often curable men or women. In his nineteen months at Bicêtre he carefully watched each patient and listened to the story of each life and symptom so he could track the "natural history of the disease" as well as the patient's life history. For he conceived of each illness as the intersection of two ongoing developments, in the disease and in the person. He was also keenly aware of the interaction of psychologic with physical complaints and published three books explicating his nosologic method, his ideas on mental alienation, and his experience at the sickbed. He taught his approach to clinical medicine for thirty years while living at Salpêtrière Hospice among his patients, most of whom were indigent. That message of a humane and patient-centered public medicine conveys the image of Pinel that should replace the myth.

Notes

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1. M. Gourévitch, "Pinel père fondateur, mythes et rélites," *L'évolution psychiatrique*, lvi (1991), 595–602; the citation is on p. 602.

2. There is as yet no modern, critical biography of Pinel in the context of the Revolution. The standard work is R. Semelaigne, *Philippe Pinel et son oeuvre au point de vue de la médecine mentale* (Paris: Imprimeries réunies, 1888). Building blocks for a detailed study can be found in the published articles by G. Bollotte, P. Chabbert, J. Postel, and D. B. Weiner. See the Bio-Bibliographic Note on Pinel in Weiner, "Mind and Body in the Clinic: Philippe Pinel, Alexander Crichton, Dominique Esquirol and the Birth of Psychiatry" in G. Rousseau (ed.), *The Languages of Psyche: Mind and Body in Enlightenment Thought* (Berkeley: University of California Press, 1990), 331–404. For masterful brief sketches of Pinel's importance to psychiatry, see J. Delay, "Philippe Pinel à la Salpêtrière," *Médecine de France*, xcvi (1958), 1–16; and P. Chabbert, "Pinel," in *Dictionary of Scientific Biography, 10*: 611–614. The topic of the myth was recently explored in J. Gortais, "Le rôle du mythe de Pinel dans l'organisation et le fonctionnement de la psychiatrie du 19ème siècle," *Psychiatrie française*, xi (1980), 77–82; and in J. Postel, "Philippe Pinel et le mythe fondateur de la psychiatrie française," *Psychanalyse à l'Université*, iv (1979), 197–244. For the historic context of this story, see D. B. Weiner, *The Citizen-Patient in Revolutionary and Imperial Paris* (Baltimore: Johns Hopkins University Press, 1993), ch. 9.

3. Gladys Swain's work is of fundamental importance in the recent reinterpretation of Pinel's meaning and influence. See G. Swain, *Le sujet de la folie: Naissance de la psychiatrie* (Toulouse: Privat, 1977), especially Part 3, "Les chaînes qu'on enlève;" idem, "La nouveauté du *Traité*

médico-philosophique et ses racines historiques,'' *Information psychiatrique*, liii (1977), 463–476; idem, ''Une logique de l'inclusion: Les infirmes du signe,'' *Esprit*, v (1982), 61–75; idem, ''Du traitement moral: Remarques sur la formation de l'idée contemporaine de psychothérapie,'' *Confrontations psychiatriques*, xxvi (1986), 19–40.

4. S. Pinel, '''Sur l'abolition des chaînes des aliénés,' par Philippe Pinel, membre de l'Institut, etc.; Note extraite de ses cahiers et communiquée par M. Pinel fils,'' *Archives générales de médecine*, ii (1823), 15–17.

5. It is curious to discover that Gevingey is a village not far from Géruge where Madame Pinel was born. She may have named the ex-prisoner ''Chevingé'' when she watched him become her husband's protector and her sons' playmate.

6. S. Pinel, ''Bicêtre en 1792. De l'abolition des chaînes,'' *Mémoires de l'Académie royale de médecine*, v (1836), 31–40.

7. E. Peyriller, ''Le geste de Pinel,'' *Presse médicale*, (1950), no. 59: 1027–1028.

8. These problems are analyzed in D. B. Weiner, ''Philippe Pinel, père: Deux générations en conflit,'' *Perspectives psychiatriques*, xcvi (1984), 100–103; and idem, ''Trois moments-clé dans la vie de Philippe Pinel,'' *Comptes-rendus du 27ème congrès international d'histoire de la médecine* (Barcelona: Acadèmia de Ciencies Médiques de Catalunya i Balears, 1981), I, 154–161.

9. J. E. D. Esquirol, *Des établissements des aliénés en France et des moyens d'améliorer le sort de ces infortunés, mémoire présenté à S. E. le ministre de l'intérieur en septembre 1818* (Paris: Huzard, 1819) reprinted in J. E. D. Esquirol, *Des maladies mentales considérées sous les rapports médical, hygiénique et médico-légal* (Paris: Baillière, 1838) [Reprint, Paris: Editions Frénésie, 1989], 2: 134–150.

10. Ibid., 157.

11. The argument and sources for this interpretation are indebted to Gladys Swain.

12. P. Chabbert, ''Les origines familiales de Philippe Pinel,'' *Histoire des sciences médicales*, xi (1977), 13–18.

13. Personal communication by the abbé Bernard Desprats, archivist of the bishop of Albi, summer 1992.

14. D. B. Weiner, ''Philippe Pinel, clerc tonsuré,'' *Annales de la Société médico-psychologique*, cxlix (1991), 169–173. The article ''tonsure'' in Diderot's *Encyclopédie*, written at about the time Pinel underwent the procedure, tells us that it had become a mere formality. The abbé Desprats confirmed this view.

15. Georg Wilhelm Friedrich Hegel, *Encyclopedie der philosophischen Wissenschaften im Grundrisse (1830)*, edited by F. Nicolin and O. Poggeler (Hamburg: Meiner, 1969), 338.

16. D. B. Weiner, ''Les sources scientifiques de l'aliénisme: L'expertise de Philippe Pinel en histoire naturelle,'' *Proceedings of the XXXII International Congress for the History of Medicine* (Antwerp, Belgium, 1991), 771–780.

17. P. Pinel, *Nosographie philosophique, ou Méthode de l'analyse appliquée à la médecine* (Paris: Brosson, 1798, 2nd ed., 3 vols., 1802–1803, 3rd. ed., 1807, 4th ed. 1810, 5th ed., 1813, 6th ed., 1818).

18. P. Pinel, *Traité médico-philosophique sur l'aliénation mentale ou la manie* (Paris: Caille et Ravier, 1800). Pinel published a second, much enlarged, edition entitled *Traité médico-philosophique sur l'aliénation mentale* (Paris: Brosson, 1809).

19. P. Pinel, *La médecine clinique rendue plus précise et plus exacte par l'application de l'analyse ou Recueil et résultat d'observations sur les maladies aigües, faites à la Salpêtrière* (Paris: Brosson, Gabon, et Cie., 1802, 2nd ed., 1804, 3rd ed., 1815).

20. L. Castel, *Analyse critique et impartiale de la Nosographie philosophique de Ph. Pinel* (Paris: n.p., An VII [1798–1799]).

21. The other professors demoted to honorary status were the surgeons François Chaussier, Antoine Dubois, A. M. Lallement, and Philippe Joseph Pelletan; the medical inspector Nicolas

René Dufriche Desgenettes; the botanist Antoine Laurent de Jussieu; the pharmacists Nicolas Deyeux, and Nicolas Louis Vauquelin; the former dean Jean Jacques Leroux; and the author and hygienist Moreau de la Sarthe.

22. E. Geoffroy St. Hilaire, ''Discours aux funérailles de Philippe Pinel à l'Académie royale des sciences,'' 27 octobre 1826. Archives, Muséum d'histoire naturelle, b 4143.

23. E. Pariset, ''Eloge de Philippe Pinel,'' lu à la séance du 28 août 1827, *Histoire des membres de l'Académie royale de médecine*, 2 vols. (Paris: Baillère, 1845), I, 209–259.

24. Anon., ''Notice'' in the *Archives générales de médecine*, xiii (1827), ''Variétés,'' 623–628.

25. G. Dupuytren, ''Notice sur Philippe Pinel,'' *Journal des Débats*, 7 novembre 1826, 14–15.

26. France. Ministère de l'instruction publique. *Procès-verbaux et rapports du comité de mendicité de la Constituante, 1790–1791*, edited by C. Bloch and A. Tuetey (Paris: Imprimerie nationale, 1911), 604.

27. D. B. Weiner, ''Philippe Pinel's 'Memoir on Madness' of 11 December 1794: A Fundamental Text of Modern Psychiatry,'' *American Journal of Psychiatry*, cxlix (1992), 725–732.

28. Pussin writes ''Prairial An V'' [that is, May–June 1797] in a document in the Archives nationales, 27 A.P. (8), doc. 2. See D. B. Weiner, ''The Apprenticeship of Philippe Pinel: A New Document, 'Observations of Citizen Pussin on the Insane,' '' *American Journal of Psychiatry*, cxxxvi (1979), 1128–1134.

29. P. Pinel, *Traité médico-philosophique*, 2ème ed. Introduction, xxxi. I believe that the discrepancy between Pussin's and Pinel's dates is due to an error by Pinel: Pussin wrote down the date in the very year of its occurrence, Pinel mentioned it twelve years later.

30. Seine. Conseil général administratif des hôpitaux et hospices civils. *Rapports au conseil général d'administration des hospices civils de Paris sur les objects confiés à sa direction* (Paris: Imprimerie des hôpitaux, An XI), 82, 85.

31. Seine. Conseil général administratif des hôpitaux et hospices civils. *Rapport au conseil général d'administration des hôpitaux et hospices civils de Paris* (Paris: Imprimerie des hôpitaux, 1816), 184.

32. G. L. Bayle, ''Considérations sur la nosologie, la médecine d'observation et la médecine pratique'' (Paris: Thèse médecine, 1802).

33. This paragraph is indebted to A. Rousseau, ''Gaspard Laurent Bayle (1774–1816), le théoricien de l'Ecole de Paris,'' *Clio medica*, vi (1971), 205–211.

34. See ''Gaspard Laurent Bayle'' in A. L. J. Bayle and A. J. Thillaye (eds.), *Biographie médicale par ordre chronologique*, 2 vols. (Paris: Delahaye, 1855), 2, 884–899.

35. In 1802, for example, more than 800 students registered for Pinel's course in ''internal pathology'' at the medical school. AN, France. Affaires culturelles. *Registres d'inscriptions des élèves à l'Ecole de santé*, AJ 16, 6418.

36. F. J. V. Broussais, *Recherches sur la fièvre hectique, considérée comme dépendante d'une lésion d'action des différents systèmes, sans vice organique* (Paris: Thèse médecine), An XI; and idem, *Histoire des phlegmasies ou inflammations chroniques, fondée sur de nouvelles observations de clinique et d'anatomie pathologique: ouvrage présentant un tableau raisonné des variétés et des combinaisons diverses de ces maladies avec leurs différentes méthodes de traitment*, 2 vols. (Paris: Gabon: 1808).

37. E. H. Ackerknecht, *Medicine at the Paris Hospital, 1794–1848* (Baltimore: Johns Hopkins University Press, 1969), 62. See also idem, ''Broussais or A Forgotten Medical Revolution,'' *Bulletin of the History of Medicine*, xxvii (1953), 320–343.

38. A. L. J. Bayle, *Recherches sur les maladies mentales* (Paris: Thèse médecine, 1822).

39. A. L. J. Bayle, *Centenaire de la thèse de Bayle*, 2 vols. (Paris: Masson, 1922), I, 47.

40. Ackerknecht, *Medicine at the Paris Hospital*, 170–171.

41. G. Cuvier, ''Eloge historique de Pinel, lu le 11 juin 1827 à l'Académie des sciences,''

Mémoires (Paris: Académie des sciences, 1830), 2nd ser. *9*: ccxxi–cclx. Georges Bollotte has documented the numerous inaccuracies in the materials that Scipion handed to Cuvier, preserved in the archives of the Institut de France. G. Bollotte, ''Documents sur Philippe Pinel, recueillis et commentés,'' *Information psychiatrique*, xliv (1968), 823–841.

42. E. Pariset, ''Éloge de Philippe Pinel, lu à la séance du 28 août 1827'' in *Histoire des membres de l'Académie royale de médecine*, 2 vols.-in-1 (Paris: Baillière, 1845), *1*: 209–259.

43. I. Violet, *Le mythe de la libération des aliénés de Bicêtre par Philippe Pinel pendant la révolution française* (Paris: UER d'histoire, Paris IV, 1898). For the letter commissioning the painting, see AN F 21 47, 5ème série, dossier 52. These details are given in Violet, *Le mythe*, 103.

44. *Gazette des hôpitaux civils et militaires*, xxii (1849), 459.

45. He may well have chosen the artist, Catherine Sophie Céleste Grandsire (who married François Heussée in 1853). The only thing known about her is that she exhibited two paintings in the Salon of 1845.

46. Seine. Conseil général administratif des hôpitaux et hospices de Paris. *Comptes généraux des hôpitaux et hospices civils, enfants abandonnés, secours à domicile et direction des nourrices de la Ville de Paris. Recette, dépense, population* (Paris: Imprimerie des hôpitaux, An XIII), 194.

47. It is surprising that a learned physician should have written a long and detailed article about laundry. See P. Pinel, ''Reflexions sur la buanderie, comme objet d'économie domestique et de salubrité, et application de ces principes à un établissement à l'île du pont de Sève, ''*La médecine éclairée par les sciences physiques*, ii (1791), 12–21.

48. Société de médecine de Bruxelles, *Recueil des Observations de Médecine, Chirurgie et Pharmacie, depuis sa réorganisation et dont elle a arrêté l'impression pour l'année 1808* . . . (Bruxelles, Weissenbruch, 1808), liv.

49. *Les souvenirs du père Richard*, Bibliothèque historique de la Ville de Paris. MS C P 5318, fol. 62–63, quoted in P. Bru, *Histoire de Bicêtre* (Paris: Progrès médical, 1890), 165, n.1. See also L. Boulle, ''Bicêtre dans la tourmente révolutionnaire,'' *Bulletin de la société de l'histoire de Paris et de l'Isle de France*, cxvi (1989), 309–329.

50. Seine. . . . *Comptes généraux*, 194.

51. Designed by A. Spitz and engraved by A. Cottet.

52. ''Centenaire de la mort de Pinel et de la naissance de Vulpian,'' *Annales médico-psychologiques*, lxxxv (1927), and ''La condition du malade mental en France, de Pinel à nos jours,'' Ibid., cxxxiv (1976).

53. T. Gineste and R. Mises, ''Le statut fait à l'enfant malade mental, la place de la controverse entre Pinel et Itard,'' Ibid., 73–81.

54. ''Inauguration de la statue de Pinel'' in A. Ritti, (ed.), *Histoire des travaux de la Société médico-psychologique et éloges de ses membres*, 2 vols. (Paris: Masson, 1913–1914), I, 425–477.

55. The phrase occurs in a paper on ''Bayle and his Critics: The Reception of A. L. J. Bayle's Discovery of General Paresis by French Psychiatry,'' presented at the 63rd Annual Meeting of the American Association for the History of Medicine, Baltimore, Maryland 10–13 May 1990.

56. See N. J. Jewson, ''The Disappearance of the Sick-Man from Medical Cosmology,'' *Sociology*, x (1976), 225–244.

57. See M. Foucault, *Madness and Civilization: A History of Insanity in the Age of Reason* (New York: Random House, 1973), ch. 9, ''Birth of the Asylum.''

58. J. Goldstein, *Console and Classify: The French Psychiatric Profession in the Nineteenth Century* (New York: Cambridge University Press, 1987). See especially ch. 3, ''The Transformation of Charlatanism, or the Moral Treatment,'' and especially 72, n. 31.